

STRAIGHT PEPTIDES

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RECONSTITUTION GUIDE

Tirzepatide 10 mg — Mixing & Injection Instructions

Step-by-step protocol for the 10 mg vial. Supports titration through 10 mg weekly in a single 100-unit syringe.
For Straight Peptides Clients

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Stage 1: Mixing

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Mixing

Time: 5 minutes | Location: Clean, flat workspace

Tirzepatide arrives as a lyophilized (freeze-dried) powder. Reconstitute with 100 units (1 mL) of bacteriostatic water. Every unit on the insulin syringe equals 0.1 mg.

Products Needed

Bacteriostatic Water	30 mL multi-dose vial, 0.9% benzyl alcohol
Tirzepatide	10 mg lyophilized vial
Insulin Syringe (mixing)	100-unit (1 mL)
Insulin Syringe (dosing)	100-unit
Alcohol Swabs	Sterile, individually wrapped

Process

- Remove the protective seals from both vials and wipe each stopper with an alcohol swab.
- Using the 100-unit insulin syringe, draw 100 units of bacteriostatic water.
 - Pull to approximately 105 units to account for air bubbles.
- Insert the needle into the center of the Tirzepatide vial. Do not push the plunger — the vacuum will draw the liquid in on its own.
- Once mixed, gently roll the vial between your fingers for one full minute.
- Refrigerate the vial. After two hours refrigerated, it is ready for injection.

Critical Warning

Do not shake the vial. Shaking damages the peptide structure and reduces potency.

Storage Requirement

*Keep the reconstituted Tirzepatide vial refrigerated between 2°C–8°C (36°F–46°F).
Stable for up to 30 days refrigerated.*

Stage 2: Dosing Math

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Dosing Math

Time: 1 minute | Reference only

At 10 mg in 100 units of BAC water, every unit on the insulin syringe equals 0.1 mg of Tirzepatide.

25 units	2.5 mg (starting dose)
50 units	5.0 mg
75 units	7.5 mg
100 units	10.0 mg (full vial in one syringe)

Stage 3: Titration Schedule

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Titration Schedule

Standard 6-month weight-loss escalation

Hold each dose for four weeks minimum before titrating up. Most clients find their maintenance dose between 5 mg and 10 mg weekly. Doses above 10 mg require an upgrade to a 20 mg or 30 mg vial.

Month 1 (Weeks 1–4)	2.5 mg weekly — starting dose
Month 2 (Weeks 5–8)	5.0 mg weekly — possible maintenance dose
Month 3 (Weeks 9–12)	7.5 mg weekly — possible maintenance dose
Month 4 (Weeks 13–16)	10.0 mg weekly — ceiling for 10 mg vial
Month 5 (Weeks 17–20)	12.5 mg weekly — upgrade to 20 mg or 30 mg vial
Month 6+ (Weeks 21+)	15.0 mg weekly — upgrade to 20 mg or 30 mg vial

Protocol Note

Only increase your dose if you have been at the previous dose for at least four weeks and you no longer notice appetite suppression or fat loss. There is no benefit to rushing the titration.

Stage 4: Injection

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Injection

Time: 2 minutes | Location: Subcutaneous site

Process

1. Wipe the rubber stopper of the Tirzepatide vial with an alcohol swab.
2. Wipe the injection site with an alcohol swab. Common sites: abdomen (two inches from navel), outer thigh, back of arm.
3. Insert the 100-unit syringe into the center of the vial and draw your dose.
4. Insert the needle fully into the injection site at a 90-degree angle.
5. Slowly inject the medication.
6. Remove the needle and gently rub the area in a small circular motion to aid absorption.

Safety Note

Insulin needles are very small — most injections are painless. Rotate injection sites weekly to prevent skin irritation.

Protocol Reminders

- Walk daily — 20 to 30 minutes minimum.
- Drink nearly double the water you consumed before starting.
- Use electrolytes daily — sodium, potassium, magnesium.
- Eat fiber daily — fruits, vegetables, whole grains.

Critical Warning

Tirzepatide slows gastric emptying. Without daily fiber and water, constipation is likely. Hydration and fiber are non-negotiable.

Questions or Concerns?

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